Lenvatinib: differentiated thyroid cancer

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Lenvatinib ≤12 mg/day

Active medicines: Lenvatinib

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take at about the same time each day, with or without food. Swallow capsules whole with water; do not crush or break them. A pharmacist can teach suspension preparation: place up to 5 whole capsules in a small container or 20 mL oral syringe, add 3 mL water, apple juice or milk, wait 10 minutes, then stir or shake for 3 minutes. Take all the mixture, add another 2 mL liquid and take the rinse. For a feeding tube use water only. If a missed dose cannot be taken within 12 hours, skip it and take the next dose at the usual time.
- Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. electrocardiogram (ECG) when clinically indicated.
- Treat diarrhoea and vomiting promptly using the agreed plan because dehydration can damage the kidneys. Check blood pressure as taught. Report sudden severe headache, vision changes, bleeding, chest pain, severe abdominal pain or fluid leakage from an abnormal opening urgently.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____